Tucatinib + capecitabine + trastuzumab

National Cancer Control Programme (NCCP) 00936 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by intravenous (IV) infusion or a locally approved under-the-skin injection. Continue while beneficial and tolerated, with clinician-approved changes.

STEP 1

Before dispensing

Confirm blood tests, interactions, the exact tablet strengths and the plan for missed doses.

STEP 2

Taking tablets

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by the agreed IV or locally approved subcutaneous (SC) route. Continue while beneficial and tolerated, with clinician-approved changes.

STEP 3

Review before restarting

The team confirms blood results, side effects and the next prescription. Never double a missed dose or restart held treatment without advice.

STEP 4

Trastuzumab visit

Trastuzumab every three weeks with staff observation. IV treatment may use a cannula, peripherally inserted central catheter (PICC) or port; the locally approved SC alternative uses an injection under the skin; no routine take-home infusion pump.

STEP 5

Confirm the trastuzumab route

Your team confirms IV infusion or the locally approved under-the-skin injection. The visit length, observation and missed-dose instructions differ; ask before switching. The injection has no loading dose.

Missed or vomited dose

Do not take an extra dose. Follow the medicine-specific instructions and ask the team if uncertain.

Safe handling

Swallow tablets whole, keep them in their original packaging away from children, and return unused tablets to pharmacy.

Practical arrangements

Confirm who supplies the tablets, who arranges tests, who reviews results and your urgent contact route.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Sources: NCCP 00936 v1 · official regimen · eviQ · implanted port patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.